

1. CLINICAL OVERVIEW

Patient presents with **Glioblastoma Multiforme** (IDH-wildtype). The current clinical status and molecular profile indicate a **HIGH RISK** profile, requiring the following therapeutic intervention.

PATIENT NAME

Robert Miller

MEDICAL RECORD NUMBER

MRN-88291-B

CLINICAL RISK LEVEL**HIGH****GENDER / AGE**

Male / 62

DIAGNOSIS DATE

15 May 2026

WHO GRADE

WHO Grade 4

PRIMARY CLINICAL RECOMMENDATION

CONCURRENT TEMOZOLOMIDE AND RADIOTHERAPY

CLINICAL RATIONALE & EVIDENCE BASIS

- MGMT Methylation
- High tumor volume

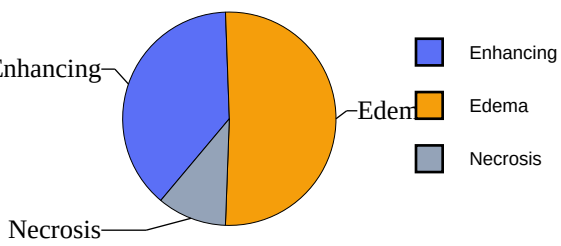
2. GENOMIC & MOLECULAR LANDSCAPE

TOTAL VARIANTS	ACTIONABLE MUTATIONS	VARIANTS OF UNKNOWN SIG. (VUS)
4,201	12	89

GENE	STATUS	CLINICAL IMPACT	TIER
IDH1	Wild-type	Aggressive phenotype	High
MGMT	Methylated	Chemo-sensitive	High

3. IMAGING & VOLUMETRIC ANALYTICS

VOLUMETRIC DISTRIBUTION**PERFORMANCE STATUS**



KPS Score

90

ECOG Score

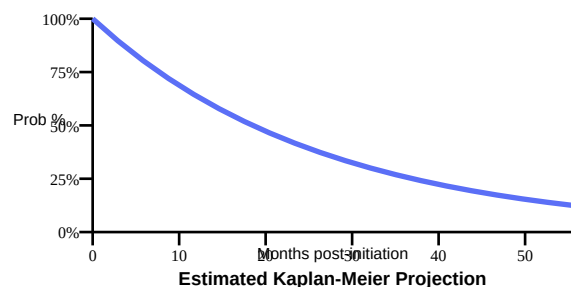
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Sphericity

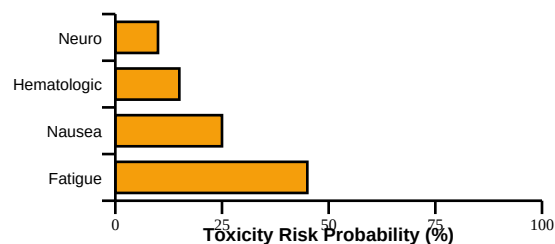
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4. ADVANCED OUTCOME PROJECTIONS

SURVIVAL PROJECTION



THERAPEUTIC TOXICITY PROFILE



5. PATIENT-REPORTED OUTCOMES (PROs)

30-Day Protocol Adherence: 91.7%

Recent Clinical Alerts:

- [WATCH] Slight decrease in energy levels reported yesterday.

IMMEDIATE CLINICAL ACTIONS

1. Initiate recommended protocol within 7-10 days.
2. Schedule baseline MRI/CT for post-treatment comparison.
3. Monitor hematological parameters weekly if systemic therapy is initiated.